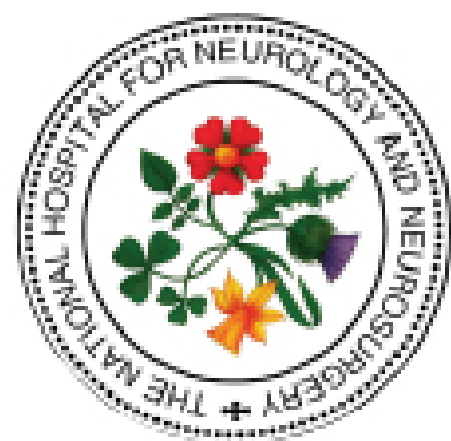
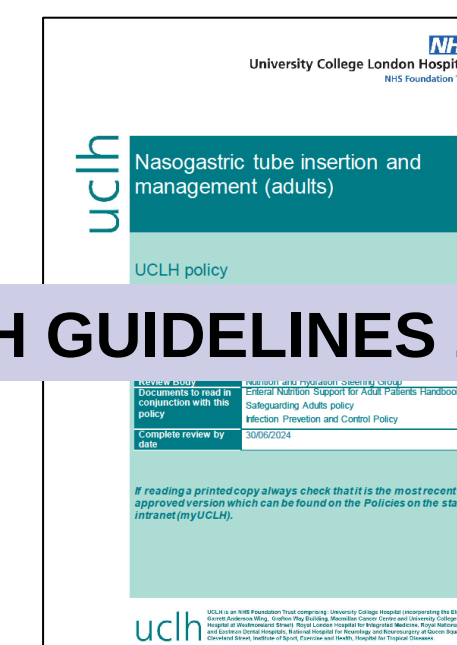




The National Hospital for Neurology and Neurosurgery Neuroscience Critical Care Unit (NCCU) (NG) ENTERAL FEEDING REGIME



UCLH GUIDELINES 2021

CONFIRM POSITION OF NGT

Confirm correct placement of Nasogastric Tube (NGT) or Orogastric Tube (OGT) prior to commencing enteral feeding as per UCLH Policy and NHNN Protocol. Start enteral nutrition as soon as possible after admission and refer all patients to Dietitian for individually prescribed feeding plan

POSITION CHECK AT LEAST DAILY

Gastric Aspirate pH ≤5.5

Check NGT still at desired NEX length
Record gastric aspirate pH

If pH >5.5

Consider stopping feed for **1 hour** to allow pH to fall – **caution if on insulin infusion**

If no aspirate within **24h** of last pH check:

- 1 Check NGT still at desired length
- 2 Consider **CEASE** triggers
- 3 Seek advice whether safe to continue feed

THIS PATIENT HAS A NASOGASTRIC TUBE (NGT) Triggers to immediately CEASE NG feed and medications until correct positioning of NGT is reconfirmed	
C	Coughing spasms / Frequent suctioning
E	Elevated respiratory rate / Difficulty breathing
A	Agitation / Drop in GCS
S	Saturation drop / Increasing O ₂ requirement
E	Elevated heart rate / Chest pain
If any CEASE symptoms are present (especially following recent insertion of NG tube) inform nurse in charge immediately. Refer to NG Protocol / Checklist and Trust Handbook 'Enteral Nutrition Support for Adults'.	

Remember

- Consider reducing opiates if not absorbing
- Use only fine bore NG tubes for feeding
- If patient admitted with wide bore tube, change to fine bore within 48 hours of admission
- Flush NGT before and after administration of drugs and after stopping feed to prevent blocking

**START FEEDING WITH OSMOLITE HP
@ 10mL/h FOR 4 HOURS
ASPIRATE AFTER 4 HOURS
Discard all aspirate**

IS ASPIRATE > 200mL?

YES

Continue feed @ **10mL/h** for further **4 hours** then aspirate

IS ASPIRATE > 200mL?

YES

Add IV **Metoclopramide** 10mg/tds
Continue at same rate and aspirate 4 hourly
Continue **Metoclopramide** for 48 hours

IS ASPIRATE > 200mL?

YES

Change **Metoclopramide** to IV **Erythromycin** 250mg/tds and continue for 48 hours
Aspirate 4 hourly and increase rate by **25mL** increments per **4 hours** to **maximum rate of 75mL/h**

IS ASPIRATE > 200mL?

YES

Is abdomen distended with absent bowel sounds?
(Consider abdominal x-ray)

NO

Continue feeding @ **10mL/h** for **24 hours** then review

YES

Put NG / OG on **free drainage** for **24 hours** (may need to change to large bore tube) and consider **IV OMEPRAZOLE 20-40mg daily**

NO

If bowel sounds present recommence feeding @ **25mL/h** and return to **STEP 2 of algorithm**

IS ASPIRATE > 200mL?

YES

TPN MUST BE CONSIDERED

Dietitian, Pharmacist, NCC Consultant must agree TPN appropriate
(Order by 10.30am from Pharmacy)

STEP 1
Continue feed @ **10mL/h** for further **4 hours** then aspirate

IS ASPIRATE < 200mL?

YES

STEP 2
Increase rate by **10mL/h** every **8 hours** until target rate reached
Aspirate 4 hourly

IS ASPIRATE < 200mL?

YES

STEP 3
Continue feeding at target rate pending Dietitian review
Aspirate 4 hourly
IF ASPIRATE > 200mL AT ANY TIME RETURN TO STEP 1 OF ALGORITHM
If on **Phenytoin** suspension rest for **2 hours** before & after administration
Adjust rate appropriately

CALCULATING TARGET RATE

Weight	Osmolite HP*	Time to Target
<49kg	40mL/h	32 hours
50-59kg	45mL/h	40 hours
60-69kg	50mL/h	40 hours
70-85kg	60mL/h	48 hours
>85kg	70mL/h	56 hours

Note: These targets are an 'out of hours' guide
Dietitian will aim to review patients within 24 hours
If stock of Osmolite HP is unavailable use Jevity in interim